



PRAKASH[®]

PATHOLOGY & RADIOLOGY PVT. LTD.

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VID : BIL1905141
Name : MRS.SAROJ
Age / Gender : 40Y(s) / Female
Ref By : SELF
Client Name :
Pt Address : L.R. ID NOT CONFIRM

UHID : TD0025063
Registered : 19-05-2025 12:12
Collected : 19-05-2025 12:13
Reported : 19-05-2025 14:29
Printed : 19-05-2025 14:29
Note :



FT3 FT4 TSH

<u>INVESTIGATION</u>	<u>RESULT</u>	<u>UNITS</u>	<u>REFERENCE RANGES</u>
METHOD	IMMUNOASSAY		
KIT	VITROS		
FT3	4.81	pmol/L	4.0 - 8.3
FT4	18.0	pmol/L	10.00-28.20
TSH	2.702	u IU/ml	0.4001-4.049
IMPRESSION	WITHIN NORMAL LIMIT (WNL)		

Note : TRAb (TSH Receptor antibody), Anti TPO, Anti Tg, TG Ab, PTH, IPTH and Calcitonin also available.

Suggested Reference Interval Pregnant Population (TSH) -

1st Trimester : 0.1298-3.120 uIU/mL (mIU/L)

2nd Trimester : 0.2749-2.652 uIU/mL (mIU/L)

3rd Trimester : 0.3127-2.947 uIU/mL (mIU/L)

Profile Interpretation :-

(I). **Patients not receiving treatment** - (A) Normal TSH with normal FT4 and FT3 - EUTHYROID.

(B) TSH (increased) with normal or decreased FT4 - HYPOTHYROIDISM.

TSH (increased) with increased FT4 - Hypothalamic - pituitary Axis (Adenoma, resistance to thyroid hormones).

(C) TSH (low) - increased FT4 and/or FT3 - HYPERTHYROIDISM.

TSH (low) with normal FT4 and/or FT3 - NTI, drugs, pregnancy Hyperthyroidism (nodular or diffuse goiter) viral pathologies, drugs, iodine.

TSH (low) with decreased FT4 and FT3 - SECONDARY HYPOTHYROIDISM, NTI.

(II) **Main causes of discrepant profiles (other than thyroid pathologies):**

Dileep

DILEEP
AST. TYPIST

Print Date : 19-05-2025 14:29 Print By: DILEEP

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(a) **Decreased TSH** - *Early stage of pregnancy.*Glucocorticoids, dopamine, bromocriptine, serotonin, opiates, DT4 triiodoacetic acid.

*Severe non thyroid illnesses (NTI, psychiatric disorders).

(b) **Moderate TSH increase** - *Dopamine antagonists and neuroleptics (metoclopramide, chlorpromazine, haloperidol domperidone, sulpiride) lithium, amiodarone.

(c) **Increased FT4** - *Amiodarone, propranolol, active acute and chronic hepatitis, DT4 (Dextrogyrol T4).

(d) **Decreased FT4** - *Kidney disorders, phenobarbital, carbamazepine, diphenyl-hydantoin.

(e) **Increased FT3** - *Triiodoacetic acid, DT4.

(f) **Decreased FT3** - *Fasting, cordarone, propranolol, NTI, cirrhosis.

Effect of age - * TSH peak during first 14 days of life. Normal <10.0 u IU/ml, borderline 10.0-20.0 u IU/ml, and

>20.0 u IU/ml hypo function.

* FT4 levels higher in newborns than in adults (with lower FT3).

* FT3 levels higher in children and adolescents than in adults.

* FT3 levels reduced in the elderly.

*****END OF THE REPORT*****



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